

BURTON HOSPITALS NHS FOUNDATION TRUST
REFERRAL FORM FOR PATIENTS WITH
SUSPECTED CANCER

HOSPITAL USE Date Fax Received

Consultant

Appointment Date Time

Patient Information

Send via e-Referral Service

Patient NHS No: **Not known**

Surname: **Mickey**

Forename: **Mouse**

Address: **St Chads**
Cannock
WS7 0EW

Date of Birth: **10-Feb-2000**

Age: **26y**

Hospital No

Patient's Daytime Telephone No:

Patient's Evening Telephone No:

Patient's Mobile Telephone No:

Patient is happy for a message to be left? YES ☐ NO ☐

Patient gender: **Male**

Does the patient know that cancer is a possibility?

YES ☐ NO ☐

Does the patient have any special needs? YES ☐ NO ☐

- Key Relative/Carer

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-Ambulance transport required?

YES ☐ NO ☐

Sensory deficit? YES ☐ NO ☐

If yes please specify

Interpreter (specify)

- Physical impairment (detail)

-Is the patient an overseas visitor? YES ☐ NO ☐

Has the patient previously visited this hospital? YES ☐ NO ☐

Referring GP: **n**

PCG Code:

Registered GP: **WARD, James (Dr)**

GP Tel No: 01543 682654

GP FAX NO.: n/a- via email:
darwinmedical.practice@nhs.net

Practice Stamp:

Darwin Medical Practice
Greenwood Health Centre
Lichfield Road
Burntwood
Staffordshire
WS7 0AQ
Tel: 01543 682654

Email: darwinmedical.practice@nhs.net

Date of Decision to Refer: **7 May 2026**

Signature of GP: nn

Tumour Site	1. Lung ☐	4. Breast (use Derby form) ☐	7. Haematological ☐	10. Brain/CNS (DO NOT USE) ☐
(Please Specify)	2. Upper GI ☐	5. Gynaecological ☐	8. Skin ☐	11. Sarcomas (USE DERBY FORM) ☐
	3. Lower GI ☐	6. Urological ☒	9. Head & Neck ☐	

I confirm that the patient fulfils the following criteria for urgent referral for suspected cancer (use coding chart)

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Clinical examination/investigations undertaken: NB: for Prostate referrals, please ensure a PSA and eGFR done within 3 months for an MRI to be ordered.

n

Information:- (please provide relevant and concise medical history and co-morbidities)

PMH and current medication list is attached with the referral.

For hospital use:

Date received Time received Date 1st appt booked Date of 1st appt

Form completed adequately? Yes ☐ No ☐ Referral criteria appropriately applied? Yes ☐ No ☐

NHS No:
MICKEY, Mouse (Mr) (dob 10-Feb-2000)

Problems

Active

Date	Problem	Associated Text
15-Apr-2026	Intermittent claudication	
09-Apr-2026	Cough	-
09-Apr-2026	Urinary tract infection, site not specified	confirmed on MSU
02-Apr-2026	Referral to mental health team	
02-Apr-2026	Review of patient laboratory test report	ACTION REQUIRED
19-Feb-2026	Has ReSPECT (Recommended Summary Plan for Emergency Care and Treatment)	
12-Feb-2026	Penicillin adverse reaction	
09-Feb-2026	Postcoital oral contraceptive given	
09-Feb-2026	Malignant neoplasm of prostate	
08-Jan-2026	Domestic abuse	
12-Dec-2025	Fatigue	
12-Dec-2025	High vaginal swab culture positive	
12-Dec-2025	Vitamin D insufficiency	
02-Oct-2025	Record contains third party information	
17-Sep-2025	Chronic kidney disease stage 5	
17-Sep-2025	Chronic kidney disease stage 2	
17-Sep-2025	Ischaemic heart disease	
12-Sep-2025	Allergy to cow's milk protein	
12-Sep-2025	Wheezing	
26-Aug-2025	Diphtheria, tetanus and acellular pertussis, inactivated polio, Haemophilus influenzae type b and hepatitis B vaccination	
26-Aug-2025	Diphtheria vaccination	
18-Aug-2025	Depression	
12-Aug-2025	Mild wheeze	
10-Jul-2025	Depot contraceptive repeated	
10-Jul-2025	Prediabetes	
30-Jun-2025	Drug compliance poor	
27-Jun-2025	Cancer care review	
23-Jun-2025	Cervical smear - recall delete	
22-Jun-2025	Oral contraceptive started	
22-Jun-2025	Miscarriage	
20-Jun-2025	Overdose	
20-Jun-2025	Problem behaviour	
20-Jun-2025	Seborrhoeic keratosis	
16-Jan-2025	Safeguarding concern	
30-Oct-2024	Harmful alcohol use	
28-Feb-2024	Diabetes mellitus	
28-Feb-2024	COPD - Chronic obstructive pulmonary disease	
06-Feb-2023	Essential hypertension	
09-Jan-2023	Asthma	(Grouped with Asthma annual review)
30-Nov-2022	Pre-diabetes	(Grouped with Diabetes mellitus)
05-Apr-2022	Refused consent for upload to local shared electronic record	
18-Mar-2022	Death	cardiac arrest
15-Mar-2022	Benign prostatic hyperplasia	(Replaced Malignant tumour of prostate)
25-Feb-2022	Examination of learning disabled patient	
05-Aug-2021	Type 2 diabetes mellitus	
26-May-2021	Preferred place of death: home	(Grouped with Death)
08-Apr-2021	Diabetic monitoring	(Grouped with Diabetes mellitus)
07-Apr-2021	Congestive heart failure	
03-Feb-2021	Housebound	
19-Oct-2020	Patient died in hospital	(Grouped with Death)
01-Jun-2020	Triple-negative breast cancer	
17-Apr-2020	Vulnerable adult	

Significant Past

Date	Problem	Associated Text
08-Jan-2026	Postcoital bleeding	
12-Dec-2025	Viral disease	
28-Nov-2025	Release of trigger finger	
12-Sep-2025	Loss of appetite	
12-Sep-2025	Tooth extraction	
12-Sep-2025	Excision of Morton's neuroma of peripheral nerve	
07-Jul-2025	Urinary tract infection in pregnancy	
04-Jul-2025	Infective otitis externa	
30-Jun-2025	Meticillin resistant staphylococcus aureus	
27-Jun-2025	Postcoital bleeding	
27-Jun-2025	Injury of head	
27-Jun-2025	Cystoscopic removal of ureteric stent	
26-Jun-2025	O/E - wound necrotic	
26-Jun-2025	Born by elective caesarean section	
26-Jun-2025	Born by caesarean section	
26-Jun-2025	Born by normal vaginal delivery	
23-Jun-2025	Endoscopy normal	
23-Jun-2025	Renal colic	
22-Jun-2025	Missed miscarriage	
22-Jun-2025	Normal delivery procedure	
22-Jun-2025	Echocardiogram shows normal left ventricular function	
22-Jun-2025	Haemorrhoids	
20-Jun-2025	Melanocytic naevus of skin	
20-Feb-2025	Insertion of hormone releasing intrauterine contraceptive device	
19-Nov-2024	Melanocytic naevus of skin	
04-Jan-2023	Clostridium difficile infection	
02-Mar-2022	Bell's palsy	
05-Aug-2021	Headache	(Replaced Gout)
29-Sep-2020	Premature menopause	
16-Jun-2020	Insertion of hormone releasing intrauterine contraceptive device	
17-Apr-2020	Paranoid ideation	

Medication**Acute**

Drug	Dosage	Quantity	Last Issued On
Midazolam 5mg/5ml solution for injection ampoules	2.5-5mg 4 hourly as needed	2 ampoule	09-Apr-2026
Morphine sulfate 10mg/1ml solution for injection ampoules	2.5-5 mg every 3-4 hours prn as needed max 30mg per 24 hour	5 ampoule	09-Apr-2026
Folic acid 5mg tablets	One To Be Taken Each Day	28 tablet	
Rifampicin 300mg capsules	One To Be Taken At Night	100 capsule	
Isoniazid 50mg tablets	One Or Two To Be Taken At Night	56 tablet	
Sofradex ear/eye drops (Neon Healthcare Ltd)	Two Or Three Drops Instilled Into The Affected Ear(s) Three Or Four Times Daily	8 ml	
Nitrofurantoin 100mg modified-release capsules	One To Be Taken Twice A Day	10 capsule	
Haloperidol 5mg/1ml solution for injection ampoules	1.5-2.5mg every 4 hours as needed	5 ampoule	
Doxycycline 100mg capsules	Two To Be Taken On The First Day Then One To Be Taken Each Day	8 capsule	

Colecalciferol 20,000unit capsules	take one capsule twice a week for 7 weeks then use an over the counter supplement which can be bought from supermarkets or chemists fro October to March	15 capsule
Hydroxocobalamin 1mg/1ml solution for injection ampoules	1ML	1 ampoule
Shingrix (Herpes Zoster) adjuvanted recombinant vaccine powder and suspension for suspension for injection 0.5ml vials (GlaxoSmithKline UK Ltd)	When Required	1 vial
Diamorphine 10mg powder for solution for injection ampoules	Anticipatory care as per community nursing authorisation and administration record for palliative care for pain if required	2 ampoule
Hydroxocobalamin 1mg/1ml solution for injection ampoules	Immediately	1 ampoule
Fortisip Compact liquid (Flavour Not Specified)	125ml twice a day	7000 ml
Fortisip Compact liquid (Flavour Not Specified)	125ml twice a day	7000 ml
Ciprofloxacin 0.3% ear drops	as directed	10 ml
Midazolam 5mg/5ml solution for injection ampoules	2.5-5mg 4 hourly as needed	2 ampoule
Diamorphine 30mg powder for solution for injection ampoules	Anticipatory care as per community nursing authorisation and administration record for palliative care for pain if required	2 ampoule
Hydroxocobalamin 1mg/1ml solution for injection ampoules	Immediately	1 ampoule
Salamol 100micrograms/dose inhaler CFC free (Teva UK Ltd)	One Or Two Puffs To Be Inhaled Up To Four Times A Day	200 x 200 dose
Paracetamol 500mg tablets	One To Be Taken Every 4-6 Hours Up To Four Times A Day	32 tablet
Clarithromycin 500mg tablets	One To Be Taken Twice A Day	10 tablet
Humulin I KwikPen 100units/ml suspension for injection 3ml pre-filled pens (Eli Lilly and Company Ltd)	For Subcutaneous Injection, According To Requirements	5 pre-filled disposable injection
Water for injections 10ml ampoules	as per directive	20 ampoule
Midazolam 10mg/2ml solution for injection ampoules	2.5mg PRN for agitation	10 ampoule
Morphine sulfate 10mg/1ml solution for injection ampoules	As per directive	10 ampoule
Paracetamol 500mg tablets	One To Be Taken Every 4-6 Hours Up To Four Times A Day	100 tablet
Folic acid 5mg tablets	One To Be Taken Each Day 4 months and then stop	112 tablet
Nitrofurantoin 100mg modified-release capsules	One To Be Taken Twice A Day	10 capsule
Morphine sulfate 10mg/1ml solution for injection ampoules	2.5-5 mg every 3-4 hours prn as needed max 30mg per 24 hour	5 ampoule
Hydroxocobalamin 1mg/1ml solution for injection ampoules	1ML	1 ampoule

Dressit sterile dressing pack with medium/large gloves (Richardson Healthcare Ltd)	10	10 pack
Dihydrocodeine 30mg tablets	One To Be Taken Four Times A Day When Required	14 tablet
Phenoxymethylpenicillin 250mg tablets	Two To Be Taken Four Times A Day	56 tablet
Paracetamol 500mg tablets	Two To Be Taken Every 4-6 Hours Up To Four Times A Day	30 tablet
Haloperidol 5mg/1ml solution for injection ampoules	Anticipatory care as per community nursing authorisation and administration record for palliative care	3 ampoule
Water for injections 10ml ampoules	Anticipatory care as per community nursing authorisation and administration record for palliative care	10 ampoule
Ibuprofen 400mg tablets	One or two To Be Taken Three Times A Day After Food as needed	84 tablet
Enoxaparin sodium 60mg/0.6ml solution for injection pre-filled syringes	As directed	10 pre-filled disposable injection
Ciprofloxacin 500mg tablets	One To Be Taken Twice A Day	14 tablet
Estradiol 10microgram pessaries	10?micrograms daily for 2 weeks, then reduced to 10?micrograms twice weekly	24 pessary
Zostavax vaccine powder and solvent for suspension for injection 0.65ml pre-filled syringes (Merck Sharp & Dohme (UK) Ltd)	1	1 pre-filled disposable injection
Folic acid 5mg tablets	One To Be Taken Each Day 4 months and then stop	112 tablet
Colecalciferol 20,000unit capsules	take one capsule twice a week for 7 weeks then use an over the counter supplement which can be bought from supermarkets or chemists fro October to March	15 capsule
Doxycycline 100mg capsules	Two To Be Taken On The First Day Then One To Be Taken Each Day	56 capsule

Repeat

Drug	Dosage	Quantity	Last Issued On
Atorvastatin 40mg tablets	One To Be Taken Each Day	28 tablet	25-Feb-2026
Topiramate 50mg tablets	One To Be Taken Twice A Day	120 tablet	11-Jan-2024
Gliclazide 80mg tablets	Two To Be Taken Daily With Breakfast	0.01 tablet	
Bempedoic acid 180mg tablets	One To Be Taken Once Daily	28 tablet	

Repeat Dispensing

Drug	Dosage	Quantity	Last Issued On
Ramipril 1.25mg capsules	take one daily to lower blood pressure and protect heart	56 capsule	25-Feb-2026

Allergies

Date	Description	Associated Text
12-Feb-2026	Penicillin adverse reaction	
12-Sep-2025	Allergy to cow's milk protein	
12-Aug-2025	No known allergy	
22-Feb-2023	Allergy to penicillin	
25-Dec-2021	Allergy to penicillin	

Values and Investigations (last 6 months)

Date	Description	Value	Units	Range
09-Apr-2026	Weight loss from baseline weight	5	kg	
09-Apr-2026	Average home diastolic blood pressure	85	mmHg	
09-Apr-2026	Average home systolic blood pressure	140	mmHg	
12-Dec-2025	High vaginal swab culture positive			

Coding Chart

1. LUNG CANCER - Chest x ray suggestive of lung cancer (including pleural effusion and slowly resolving consolidation). - Persistent haemoptysis in smokers/ex-smokers over 40 years of age. - High suspicion of lung cancer with normal x-ray. - History if asbestos exposure, recent onset chest pain, shortness of breath, or where chest x-ray indicates pleural effusion, pleural mass or any suspicious lung pathology. <i>For other lung symptoms consider urgent chest x-ray first.</i> REFER IMMEDIATELY PATIENTS WITH - Signs of superior vena cava obstruction (swelling of the face/neck with fixed elevation of jugular venous pressure). - Stridor.	CODE 1 A 1B 1C 1D	3. LOWER G.I. CANCER All ages - A definite palpable right-sided lower abdominal mass. - A definite palpable rectal (not pelvic) mass. Aged 40 and over - Rectal bleeding WITH a change in bowel habit to looser stools and/or increased frequency persistent for >=6 weeks. Over 60 years - Rectal bleeding persistently WITHOUT anal symptoms or change in bowel habit. - Change of bowel habit to looser stools and/or increased frequency WITHOUT rectal bleeding and persistent for >=6 weeks. Any age - Iron deficiency anaemia WITHOUT an Obvious cause (Hb <11g/100ml in men or <10g/100ml in postmenopausal women. Please ensure that a full blood count, abdominal or rectal examination has been carried out as appropriate prior to referral.	CODE 3A 3B 3C 3D 3E 3F	5. GYNAECOLOGY CANCER - Clinical features suggestive of cervical cancer on examination. - Vulval bleeding due to ulceration or an unexplained vulval lump. - Palpable pelvic or abdominal mass not obviously fibroids or of gastrointestinal or urological origin. - Postmenopausal bleeding where patient not on hormone therapy, or where patient taking tamoxifen. - Persistent intermenstrual bleeding and negative pelvic examination. - On HRT with unexpected or prolonged bleeding persisting for more than 6 weeks after stopping HRT.	CODE 5A 5B 5C/D 5E 5F 5G
2. UPPER G.I. CANCER - Dysphagia – food sticking on swallowing (any age). - Dyspepsia at any age combined with one or more of the following ‘alarm’ symptoms - weight loss - proven anaemia - vomiting - epigastric mass - dysphagia - chronic gastrointestinal bleeding - suspicious barium meal result - Unexplained persistent recent onset dyspepsia in a patient aged 55 years or more. - Obstructive jaundice. - Upper abdominal mass without dyspepsia. - Unexplained upper abdominal pain and weight loss, with or without back pain. CONSIDER urgent referral for patients with: - Unexplained worsening of dyspepsia combined with: - Barrett’s oesophagus - Peptic ulcer surgery over 20 years ago - Known dysplasia, atrophic gastritis - Intestinal metaplasia. - Persistent vomiting and weight loss in the absence of dyspepsia. - Unexplained weight loss or iron deficiency anaemia in the absence of dyspepsia.	2A 2B 2C 2D 2E 2F 2G 2H 2I	4. BREAST CANCER - Female patients with a lump: - any age – with a discrete, hard lump <u>with fixation</u>, with or without skin tethering - aged >=30 years – with a discrete lump that persists after their next period or presents after menopause. - Aged <30 years – with a lump that enlarges, is fixed and hard, or in whom other reasons for concern apply e.g. family history. - Previous breast cancer patients – with a further lump or suspicious symptoms. - Patients with one of the following nipple problems: - unilateral eczematous skin or nipple change that does not respond to topical treatment. - nipple distortion of recent onset. - spontaneous unilateral bloody nipple discharge. - Male patients, aged >=50 years with a Unilateral, firm subareolar mass with or without nipples distortion or associated skin changes.	4A 4B 4C 4D	6. UROLOGICAL CANCER - Macroscopic Haematuria in adults. - Microscopic Haematuria in adults over 50 years. - Swelling or mass in the body of the testis. - Abdominal mass identified clinically or on imaging, thought to arise from urinary tract. - Recurrent or persistent urinary tract infection associated with haematuria in adults aged >=40 years. - An elevated age specific PSA in men who are not significantly compromised by other co-morbidities. - A high PSA (>20ng/ml) in men with a clinically malignant prostate. (PSA result should accompany referral) - Any suspected penile cancer. - With symptoms and high PSA levels.	6A 6B 6C 6D 6E 6F 6G 6H 6I

Coding Chart

7. HAEMATOLOGICAL CANCERS • Constellation of 3 or more of the following symptoms: fatigue, night sweats, weight loss, itching, breathlessness, bruising, recurrent infections, bone pain, fever, bleeding, alcohol-induced pain, abdominal pain, lymphadenopathy, splenomegaly. • Persistent unexplained splenomegaly. REFER IMMEDIATELY • Patients with a blood count/film reported as acute leukaemia. • Patients with special cord compression or renal failure suspected of being caused by myeloma.	CODE 7A 7B	9. HEAD AND NECK CANCERS Urgent Referral • An unexpected persistent sore or painful throat. • Unilateral unexplained pain in the hand and neck area >4 weeks associated with otalgia but normal otoscopy. • Unexplained ulceration of oral mucosa or mass persisting > 3 weeks. • All red or red and white patches of the oral mucosa that are painful and swollen/bleeding. • Unexplained neck lump of recent onset or previous undiagnosed lump changing over 3-6 weeks. • Unexplained persistent swelling in parotid or submandibular gland. • Thyroid swelling associated with: - A solitary nodule increasing in size. - A history of neck irradiation. - Family history of endocrine tumour. - Unexplained hoarseness or voice changes. - Cervical lymphadenopathy. - Pre-pubertal patient. - Patient aged >= 65 years. REFER IMMEDIATELY • Patients with symptoms of tracheal compression including stridor due to thyroid swelling.	Refer to ENT ENT/OMF OMF OMF ENT/OMF ENT/OMF ENT	CODE 9A 9B 9C 9D 9E 9F 9G	10. BRAIN TUMOURS • Progressive neurological deficit developing over days to weeks (e.g. weakness, sensory loss, dysphasia, ataxia). • New or recent onset seizures. • Headaches or mental changes. • Cranial nerve palsy, unilateral sensorineural deafness • Headaches of recent onset accompanied by features if raised intracranial pressure eg vomiting, drowsiness, post-related headache, pulse-synchronous tinnitus, blackout, change in personality or memory, a new qualitatively different/unexplained headache that becomes progressively severe. Consider urgent referral for: • Rapid progression of subacute focal neurological deficit, unexplained cognitive impairment, behavioural disturbance or slowness, personality changes confirmed by a witness for which there is no reasonable explanation.	CODE 10A 10B 10C 10D 10E 10F									
8. SKIN CANCERS NB – EXCISION IN PRIMARY CARE SHOULD BE AVOIDED Melanoma • With a lesion suspicious of melanoma, that is changing and/or scoring 3 points or more from the features below (based on major features scoring two points each and minor features scoring one point each) :-<table><tr><td>Major Features</td><td>Minor Features</td></tr><tr><td>Change in size</td><td>Large diameter =>7mm</td></tr><tr><td>Irregular shape</td><td>Inflammation</td></tr><tr><td>Irregular colour</td><td>Oozing</td></tr><tr><td></td><td>Change in sensation</td></tr></table> • With unexpected histological diagnosis of Melanoma. SCC • Non-healing keratinising or crusted tumours >1cm with significant induration on palpation and documented expansion over 8 weeks. • With unexpected histological diagnosis of squamous cell carcinoma. • Who have had an organ transplant with new/growing cutaneous lesions. Visit www.sunsmart.org.uk for pictorial guidance.	Major Features	Minor Features	Change in size	Large diameter =>7mm	Irregular shape	Inflammation	Irregular colour	Oozing		Change in sensation	8Ai 8Aii 8Bi 8Bii 8C				
Major Features	Minor Features														
Change in size	Large diameter =>7mm														
Irregular shape	Inflammation														
Irregular colour	Oozing														
	Change in sensation														
11. BONE CANCER AND SARCOMAS • A palpable lump that is: - Size > 5 cms. - Painful. - Increasing in size. - Deep to fascia, fixed or immobile. - Recurrence after previous excision. • Patients with radiological suspicion of a bone cancer. REFER FOR IMMEDIATE X-RAY PATIENTS WITH SUSPECTED SPONTANEOUS FRACTURE.						11A 11A1 11A2 11A3 11A4 11A5 11B									

